A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 April 2026

Dr Claire Veith
East Brunswick Medical Centre

Dear Dr Veith,

Re: Ms Louise McNab
DOB: 1969-11-09
Address: 7 Woolacott Street, COBURG, VIC, 3058

Summary

- **Recent presentation to emergency department with acute abdominal pain, now resolving.**
- **Diaphragmatic eventration noted on imaging; for second surgical opinion.**
- **Incidental finding of a large adipose lesion for further evaluation via MRI and specialist review at Peter MacCallum Cancer Centre.**
- **Surveillance gastroscopy for coeliac disease scheduled for late 2026**
- **No action required by you.**

I had the pleasure of conducting a review with Louise McNab. Thank you for re-referring her. As you are aware, she recently presented to the Royal Melbourne Hospital emergency department with an episode of acute, intense abdominal pain. She described the pain as starting suddenly and increasing in severity over several hours. While initial analgesia was ineffective, the pain has since transitioned into intermittent spasms lasting 10 to 15 seconds and is improving with the use of buscopan and paracetamol.

During her admission, imaging identified a diaphragmatic defect. While there was some initial suggestion of a diaphragmatic hernia, the surgical fellow at Royal Melbourne Hospital has reviewed the films and suspects that this might not definately be the case. They have noted that this finding is unlikely to explain her acute presentation. Louise is not currently experiencing significant reflux, regurgitation, or exertional breathlessness, which are often associated with such defects.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Furthermore, imaging revealed a significant fatty lesion in the abdomen. While it has been suggested this may be a lipoma, its size warrants further investigation to exclude of other pathology such as sarcoma. The team at Royal Melbourne Hospital is currently arranging an MRI and has initiated a referral to Peter MacCallum Cancer Centre for a specialist opinion.

Regarding her gastrointestinal health, Louise reports that her bowel habits are currently stable, with solid stools passed two to three times daily. Her lymphocytic colitis and coeliac disease appear to be symptomatically not a major issue.

Impression and Plan

I have reassured Louise that the current investigations by the surgical team are appropriate. To provide further clarity regarding the diaphragmatic eventration, I will refer her to Professor Matthew Read for a second surgical opinion to determine if repair is indicated to prevent future pulmonary compromise or symptomatic progression.

In relation to her coeliac disease, Louise is due for a surveillance gastroscopy two years post-diagnosis. I have planned to perform this procedure in the second half of this year. If Mr Read determines that any surgical intervention or further endoscopic evaluation is required, we will look to coordinate these procedures. We will not repeat the colonoscopy at this stage as there is no clinical indication for surveillance of her lymphocytic colitis, given the lack of increased malignancy risk and her current symptomatic control. I will review her again following her surgical consultations and MRI results.

Dear Matthew I would be grateful if you might offer Louise an appointment for an opinion.I would be grateful if this letter could be used for medicare purposes. Louises phone number is 0425722658

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Prof Matthew Read

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